

MAT in Primary Care Readiness Checklist

Primary Care Clinic – Pre-Launch Workflows

1. Leadership, Scope, & Governance

- ☐ Identify clinical lead/champion for MAT program
 - ☐ Define clinic scope of MAT services
 - Induction vs. maintenance
 - Office-based vs. home induction
 - ☐ Establish clinical escalation pathways
 - Psychiatry consultation
 - Higher level of care (IOP, OTP, residential)
 - ☐ Confirm DEA registration and ability to prescribe buprenorphine
 - ☐ Update clinic policies to include OUD as a chronic disease model
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2. Regulatory & Legal Readiness

- ☐ Review state-specific MAT regulations and payer requirements
 - ☐ Develop written MAT policies & procedures
 - ☐ Create informed consent document
 - ☐ Create Substance Use Disorder (SUD) treatment agreement
 - ☐ Ensure HIPAA and 42 CFR Part 2–aligned documentation practices
 - ☐ Define termination / transfer of care workflow (non-punitive)
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3. Clinical Protocols (Standardized & Written)

- ☐ OUD diagnostic criteria and assessment protocol
- ☐ Buprenorphine induction protocol
- ☐ Dose titration and stabilization guidelines

- ☐ Missed appointment protocol
 - ☐ Relapse / positive UDS response protocol
 - ☐ Diversion concern response protocol
 - ☐ Tapering and transition criteria (patient-driven)
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4. Monitoring & Safety Systems

- ☐ Urine Drug Screen (UDS) standing order
 - At every visit or defined cadence
 - ☐ UDS panel standardized (including buprenorphine ± fentanyl)
 - ☐ PDMP review workflow
 - Defined frequency (e.g., every visit)
 - Clear documentation location in EHR
 - ☐ Pill/film count process
 - Routine and random checks
 - ☐ Naloxone prescribing & education workflow
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5. EHR & Documentation Build

- ☐ MAT-specific intake templates
 - ☐ Follow-up visit templates
 - ☐ Smart phrases/macros for:
 - PDMP review
 - UDS interpretation
 - Counseling documentation
 - ☐ Standing orders for labs and UDS
 - ☐ Tracking mechanism for:
 - Missed visits
 - UDS results
 - Contract compliance
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6. Staffing & Roles

- ☐ Identify MAT-trained prescribers
 - ☐ Define care team roles:
 - Prescriber
 - Nurse / MA (UDS, pill counts)
 - Behavioral health
 - Care coordination / referrals
 - ☐ Train staff in:
 - Stigma-informed care
 - Trauma-informed communication
 - De-escalation and motivational interviewing
 - ☐ Establish internal communication workflow for concerns
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7. Clinic Operations & Scheduling

- ☐ MAT-specific appointment slots created
 - ☐ Longer visit times for induction and early stabilization
 - ☐ No-show policy clarified for MAT patients
 - ☐ Emergency access protocol (withdrawal, lapse, lost meds)
 - ☐ Pharmacy coordination identified (preferred pharmacies)
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8. Behavioral Health & Community Integration

- ☐ Create referral list for:
 - Substance use counseling
 - Individual/group therapy
 - Psychiatry
- ☐ Identify community recovery resources:
 - NA / SMART Recovery

Virtual support groups

- ☐ Define expectations for counseling engagement (supportive, not punitive)
 - ☐ Warm-handoff process where available
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9. Quality, Safety, & Data Tracking

- ☐ Define MAT program metrics:
 - Retention rates
 - UDS adherence
 - Overdose prevention measures
 - ☐ Adverse event reporting workflow
 - ☐ Periodic chart review process
 - ☐ Continuous quality improvement (QI) plan
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10. Patient Education & Materials

- ☐ MAT patient handbook or packet
 - ☐ Written induction instructions
 - ☐ Safe storage guidance
 - ☐ Overdose prevention education
 - ☐ Clear contact instructions for urgent issues
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FINAL READINESS CHECK

- ☐ All workflows documented
- ☐ Staff trained
- ☐ EHR tools built
- ☐ Community partnerships established
- ☐ Leadership approval to launch MAT services